

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	752
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	07/2022
		Review Date:	07/2028

TRAUMA	
Patients who meet EMS Policy #402 – Patient Destination, (3) Trauma Patient Destination should be transported to a Trauma Center. Trauma patients with potential airway compromise that cannot be resolved must be transported to the closest hospital. Scene time for patients who meet Trauma Patient Destination Criteria should be kept at 10 minutes or less.	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Bleeding Control - Stop active bleeding with direct pressure, hemostatic dressings, or tourniquet. Spinal Motion Restriction (SMR) - Refer to EMS Policy #700 – General Procedures. Splint - Immobilize or splint as needed, consider splinting while transporting. Temperature - Keep patient warm and prevent hypothermia.	
ALS	
Follow BLS procedures if applicable Assess for Tension Pneumothorax and decompress the affected side as needed Obtain 4-Lead ECG – If indicated. Vascular Access - IV/IO rate as appropriate Trauma Patient Destination Criteria – two (2) large bore IV/IO access Capnography - Trauma Patient Destination Criteria.	
Specific Injury Treatments	
Amputations - If partial amputation, splint in anatomical position. Place complete amputated parts in a sealed clean and dry container or bag. Place container or bag on ice if possible. Eviscerating Trauma - Cover eviscerated organs with saline-soaked gauze. Do not attempt to replace organs in the abdominal cavity. Extremity Trauma - Apply appropriate splint/immobilization device, check neurovascular status before and after splinting/immobilizing. Flail Chest - Consider laying patient on the affected side to minimize chest wall movement, monitor for tension pneumothorax. Impaled Object - Stabilize and leave impaled object in place. If an impaled object causes airway compromise or interferes with CPR, object can be removed. Open Chest Wound - Apply commercial vented chest seal or 3-sided occlusive dressing over wound. Watch for signs of developing tension pneumothorax and treat as appropriate.	

Signatures on File

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